

Department of Pediatrics
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 2.9 Yr/M
Admission No.: anonymous	Department Unit: Pediatrics (Pedia Unit 2)	Consultant/Faculty: anonymous
Ward: N I C U	Room/Bed: In Born Unit/ I B U 2	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 30-Mar-2023 16:34	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:18

FINAL DIAGNOSIS:

TERM / AGA/MALE/PMA-43 5WK/LSCS I/V/O PREVIOUS 2 LSCS/SVT/MULTIPLE SPACE OCCUPYING LEASION IN HEART?
RHABDOMYOMA

/ ? TUBEROUS SCLEROSIS

CHIEF COMPLAINTS:

DIFFICULTY IN FEEDING
POOR FEEDING
IRRITABILITY
at day 10 of life

HISTORY OF PRESENT ILLNESS: B/O GEETA ,MALE BABY,, DELIVERED ON 13/3/23 AT 4:56 PM . THROUGH ELECTIVE LSCS I/V/O PREVIOUS 2 LSCS WITH POG 40 5 WK WITH BIRTH WEIGHT 3100G.CRIED IMMEDIATELY AFTER BIRTH WITH NORMAL TONE and 9 /10APGAR SCORE AT 1 MIN . BABY WAS APPARENTLY WELL TILL 10 DAYS OF AGE WHEN BABY DEVELOPED DIFFICULTY BREATHING, POOR FEEDING, IRRITABILITY. BABY WAS ADMITTED IN PRIVATE HOSPITAL FOR 1 DAY AND REFERRED TO RAIPUR IN VIEW OF REGULARLY IRREGULAR HEART RATE,TACHYCARDIA AND HYPOXIC SEIZURE .ON ADMISSION AT PETALS HOSPITAL BABY HAD GRUNTING, RESPIRATORY DISTRESS FOLLOWING WHICH BABY WAS INTUBATED AND AND KEPT ON VENTILATORY SUPPORT. IV FLUIDS , IV ANTIBIOTICS, IV ANTIEPILEPTICS DRUGS WERE GIVEN. IV LASIX WAS ALSO GIVEN. CHEST XRAY REVEALED CARDIOMEGALY AND 2D ECHO SHOWED MULTIPLE SPACE OCCUPYING LESIONS IN HEART WITH SEVERE MR WITH MILD MS AND MILD PAH .CBC , CRP ARE WNL, USG CRANIUM ANND ABDOMEN NORMAL. BABY HAD TACHCARDIA 220-230 FOR WHICH ADENOSINE WAS GIVEN AND FURTHER STARED WITH TAB PROPRANOLOL NEXT DAY. VENTILATORY SETTING REDUCED GRADUALLY AND BABY WAS EXTUBATED ON DAY 4 OF ADMISSION AND PUT ON O2 SUPPORT WITH NASAL PRONGS WHICH WAS WEANED OFF WITHIN 1 DAY. BABY HAD 3 MORE EPISODES OF TACHYCARDIA WHICH SETTLED WITH ADENOSINE. fUROPED DROPS,ALDACTONE WERE ADDED. CHILD HAD NO FURTHER EPISODES OF ARRYHTMIA AND WAS REFERRED TO AIIMS FOR FURTHER MANAGEMENT.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
03-Apr-2023 13:01	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.15	0.1 - 2 x10 ³ /μL
		Absolute Lymphocyte Count	6.17	2 - 16 x10 ³ /μL

		Absolute Monocyte Count	2.01	0.1 - 1.2 x10 ³ /μL
		Absolute Neutrophil Count	1.02	3 - 9 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	1.6	01 - 06 %
		HCT	38.8	33 - 66 %
		Hemoglobin	12.7	11.5 - 21.5 g/dl
		Lymphocytes	65.8	31 - 71 %
		MCH	33.3	30 - 37 pg
		MCHC	32.7	28 - 38 g/dL
		MCV	101.8	86 - 126 fL
		Monocytes	21.5	2 - 10 %
		Neutrophils	10.9	20 - 60 %
		Platelet Count	540	160 - 500 x10 ³ /μL
		RBC	3.81	3 - 6.3 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	15.1	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	9.37	5 - 22 x10 ³ /μL
	Liver Fuction Test (Lft)	Albumin	4.3	4.0 - 5.0 gm/dL
		ALP	318	50 - 116 U/L
		ALT	37	7 - 41 U/L
		AST	47	12 - 38 U/L
		Direct Bilirubin	0.39	0.1 - 0.4 mg/dl
		globulin	2.2	2.0 - 3.5 gm/dL
		Indirect Bilirubin	0.86	0.20 - 0.90 mg/dl
		Total Bilirubin	1.25	0.3 - 1.3 mg/dl
		Total Protein	6.5	6.7 - 8.6 gm/dL
	Rft	Chloride	97	102 - 109 mmol/L
		Creatinine	0.59	0.9 - 1.3 mg/dl
		Potassium	4.83	3.5 - 5 mmol/L
		Sodium	134	135 - 145 mmol/L
		Urea	25	15.0 - 39.0 mg/dl
01-Apr-2023 13:48	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	APTT HEADER	20.1	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.33	0.1 - 2 x10 ³ /μL
		Absolute Lymphocyte Count	6.34	2 - 16 x10 ³ /μL

Absolute Monocyte Count	0.56	0.1 - 1.2 x10 ³ /μL
Absolute Neutrophil Count	3.89	3 - 9 x10 ³ /μL
Basophil	0.2	00 - 01 %
Eosinophils	3.0	01 - 06 %
HCT	41.2	33 - 66 %
Hemoglobin	13.4	11.5 - 21.5 g/dl
Lymphocytes	56.9	31 - 71 %
MCH	33.2	30 - 37 pg
MCHC	32.5	28 - 38 g/dL
MCV	102.0	86 - 126 fL
Monocytes	5.0	2 - 10 %
Neutrophils	34.9	20 - 60 %
Platelet Count	591	160 - 500 x10 ³ /μL
RBC	4.04	3 - 6.3 x10 ⁶ /μL
Red Cell Distribution Width (RDW CV)	14.9	11.6 - 14.0 %
Total Leukocyte Count(TLC)	11.14	5 - 22 x10 ³ /μL
PROTHROMBIN TIME clinical details (PT INR)	--	-
International Normalised Ratio (INR)	1.1	-
PT INR Header	10.6	09.6 - 12.2 sec
PT INR Note	Refer Reports*	-

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: TREATMENT GIVEN-

SYP. LEVIPIL 0.3 ML PO BD

TAB. PROPRANOLOL 3MG PO TDS

DROP FUROPED 0.3 ML PO BD

TAB. ALDOLACTONE 3 MG PO BD

DROP VITAMIN D 400IU PO BD

ECG MONITORING

RBS MONITORING

BP MONITORING EVERY DAY

HOSPITAL COURSE: CHILD WAS RECEIVED IN STABLE CONDITION AND WAS CONTINUED ON FUROSEMIDE,ALDACTONE AND LEVETIRACETAM. INSTITUTIONAL 2D ECHO WAS DONE WHICH WAS SUGGESTIVE OF LARGE MASS IN LA 22X20MM BUT NOT CAUSING OBSTRUCTION TO MITRAL INFLOW. SMALL MASS IN IVS AND LV LATERAL WALL. CTVS OPINION WAS TAKEN AND THROMBOTIC STATE WAS RULED OUT.CTVS ADVISED TO REPEAT 2D ECHO AFTER ONE WEEK TO LOOK FOR REGRESSION OF MASS IF ANY. IN VIEW OF CARDIAC MASS RHABDOMYOMA, TUBEROUS SCLEROSIS WAS KEPT A POSSIBILITY. USG CRANIUM IS PLANNED ON 5/4/23. NO SKIN LESIONS CURRENTLY NOTED.CHILD IS TO BE FOLLOWED UP ON OPD BASIS

CONDITION AT DISCHARGE: CHILD IS HEMODYNAMICALLY STABLE NOW

ADVICE ON DISCHARGE:



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



SYRUP. LEVIPIL 0.3 ML PO BD FOR 1 WEEK
TAB PROPANOLOL 3 MG PO TDS FOR 1 WEEK
DROP FUROPAD 0.3 ML PO BD FOR 1 WEEK
TAB. ALDOLACTONE 3MG PO BD FOR 1 WEEK
DROP VITAMIN D3 400IU PO OD CONTINUE FOR 1 YEAR

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 07-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW IN CTVS OPD AFTER 1 WEEK

DANGERS SIGN EXPLAINED, REVIEW IN PEDIATRIC OPD SOS

Summary Prepared By

anonymous

Summary Approved By

anonymous